

Public and Private Roles in Health

Health care is about 8 percent of world income, making it one of the largest industries in the global economy. On average, 60 percent of this is public spending. If this spending appears to be inappropriate, the consequences for the economy in terms of opportunity costs could be substantial. Despite all the arguments of market failures, it is not always very convincing that public should spend so much while close scrutiny reveals that most health care is a private good. It is therefore surprising to many economists that so much of it is provided, financed or regulated by the state.

In contrast to what happens in other sectors of the economy, this exaggerated public role is also most pronounced in the developed countries which are generally very market-oriented. On the contrary, the state usually finances a smaller share of health care in poor countries (Table-1 below)

Table-1: Basic patterns in national health expenditure (Data of May, 2001)

Country	% shares				Per capita expenditure in PPP \$		
	<u>THE</u> GDP	<u>PHE</u> THE	<u>PvtHE</u> THE	<u>OOPS</u> PvtHE	THE	PHE	OOPS
Bangladesh	4.5	45.4	54.6	95.0	50	23	26
China	4.2	39.4	60.6	78.9	125	49	60
India	5.5	15.3	84.7	97.3	109	17	90
Canada	9.0	69.9	30.1	56.9	2181	1527	374
UK	6.7	83.7	16.3	67.1	1399	1171	153
USA	13.0	45.5	54.6	28.2	3915	1780	603

Source: Bulletin of the WHO 2002; 80:134-142

The question is does this pattern provide models for the future development of the health sector and the public role in it, for low-income countries?

Besides consuming large resources, most health care systems are regarded as inefficient or inequitable or both; they are often described as in crisis- either needing "reforms" or having "failed". If so, they might be caused by misguided public intervention, so they could be converted by a smaller or different public role and a greater reliance on private markets.

Is there an appropriate frontier between private and public action, and a best combination of instruments for the state to use when it intervenes?

Choices for State Intervention

There are fine distinct instruments of public intervention: arranged them from the least to the greatest intrusion into private decisions, these are to-

1. Inform, which may mean to persuade, but does not require anyone to do anything. Governments do this when they publicize the health risks of smoking, or include health

and basic hygiene education in public schools. Governments also inform health care providers and suppliers of health care inputs as by conducting research and discriminating information on disease patterns and on the effects and risks of medical interventions/produces.

2. Regulate, while determine how a private activity may be undertaken. Governments sometimes regulate the medical profession by setting standards for doctors or accrediting hospitals, although these activities may also be undertaken by private bodies. And government regulation is common in the insurance industry, in the importation of medical equipment, drugs and suppliers and in the protection of food and water quality.
3. Mandate, which obligates someone to do something and (usually, though not always) to pay for it. Compliance with regulations can also imply substantial private costs; but a mandated activity is different in that it must be performed, whereas a private producer can react to regulation by choosing not to undertake the activity. Mandates are usually specified in law, which may subsequently be adumbrated by regulation. The most important mandates, in financial terms, are the requirements that employers provide health services or insurance to their employees, or contribute to social insurance funds for that purpose. Government can also impose mandates on individuals as by requiring that children entering school be immunized.
4. Finance, health care with public funds. Because mandated insurance is effectively paid for by an earmarked, involuntary contribution which is equivalent to a tax, "public" health expenditure is commonly defined to include such costs along with expenditures from public budgets. The obverse of spending public funds is to tax particular activities or goods, such as alcohol and tobacco, at least partly for health reasons.

Finally, the state may-

5. Provide or deliver services, using publicly-owned facilities and civil service staff. This is what Ministries of Health in most poor countries do; so do various governmental bodies in many countries at all income levels. Once a society has decided to finance health services with public funds, the choice arises of whether to provide them through public facilities or pay private producers to provide them. The appropriate way to consider this choice is as a standard "make or buy" decision.

A Conceptual Basis for Public and Private Roles in Health

Since what the health care sector provides to consumers and beneficiaries are specific interventions, it is useful to organize a conceptual basis according to distinctions among these activities.

The Three Domains of Health Care

While the activities that promote, protect or restore health are very heterogenous, they fall into three natural domains, corresponding to public goods, to low-cost private interventions and to

catastrophically costly private goods. These domains are constructed by classifying health-related activities along two-dimensions, as shown in Figure-1(below):

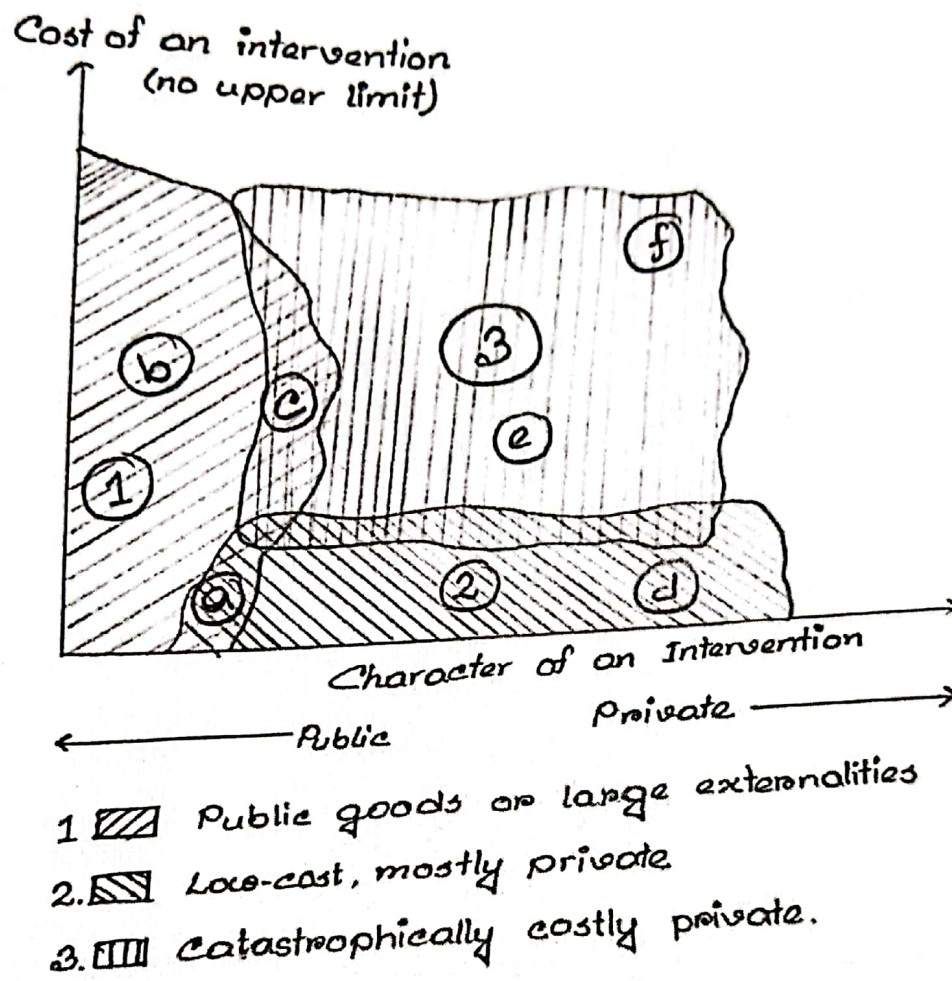


Figure -1: Three Domains of Health Care.

First, by the degree to which they are private or public goods,

Second, by how much they cost, and

Third, both dimensions refer only to characteristics of particular activities or interventions themselves, not to who consumes them or pay for them. As the descriptions of the three domains indicate, private goods are separated into low-cost and high-cost whereas all public goods constitute one domain regardless of their costs, because the issue to market failure related to such goods is independent of costs.

Examples of some specific interventions:

a. Immunization	d. Treatment of minor trauma
b. Vector Control	e. Normal obstetric care
c. Treatment of Tuberculosis	f. Surgery for cancer

A particular intervention occupies a small space in Figure-1; to indicate how different interventions would be classified into the three domains, some typical health care activities are located approximately. The cost of an intervention can vary depending on many factors, such as how widespread the intervention is; there can also be variation in the public or private good nature of an activity. Because a health system produces a variety of interventions in all three domains, it is spread widely over the space. Figure-1 therefore does not serve to compare different systems. But one can conceivably argue for a particular system based on the relative basis of public and private roles in health.